

إسم المريض رابعياً : خديجة موسى عباس سيف النصر

المحافظة : القاهرة الحى / المدينة : الزاوية الحمراء العنوان : //

رقم البطاقة : 28808102301008 رقم التليفون : 01000640254 تاريخ تسجيل الإستمارة : 21-09-2026

العمر(تاريخ الميلاد) : 10-08-1988 المهنة : حاصل على بكالوريوس

أقر انا الموقع أدناه انني لا اتمتع بأي نظام علاج مثل :- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإحصائي الاجتماعي
التوقيع: (_____)

المقر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالغده اللعابيه

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالغده اللعابيه

خطة العلاج : أورام كيماوي علاج تدعيمي وتلطيفي [6043] قسم العلاج : الاورام

التعليق على التشخيص : -----

GR1-E
روستاقی کمیٹی
نئی تحصیل
ضلع مظفر
صحة الباطن و امراض
للجنة و مدير

الاسم: د/محمد کیلانی

الاسم: د/ندی حسن

الاسم: د/نرمين رمضان

الوظيفة: ----- التوقيع:

الوظيفة: ----- التوقيع:

الوظيفة: ----- التوقيع:

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير
المستشفى.

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس

وتلتزم جهة العلاج بلائحة المجالس الطبية.

كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

----- /د ----- /د ----- /د. | ----- /د. |

قرار اللجنة

التشخيص النهائي :

خطة العلاج :

المستشفى :

جهة العلاج : --

ملاحظات : ---

قسم :

نفقات :

ختم

شعار الجمهورية

مدير عام

مديرية الشؤون الصحية

رئیس

المجلس الطبي المتخصص

--

Name: خديجة موسى عباس سيف النصر National ID: 28808102301008	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
--

A patient of salivary gland cancer for Supportive and palliative Treatment
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بطاقة تعقب الشخصية



خديجة
موسى عباس سيف النصر
اش عبده الجزار - مدينة الجندول
الزاوية الحمراء - القاهرة

٢٩٨٨/٠٨/٠٨ ٢٨٨٠٨١٠٢٣٠١٠٠٨

KN2842920

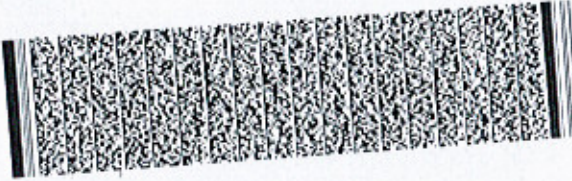
٢٠٢٤/٠٥ ٢٨٨٠٨١٠٢٣٠١٠٠٨

حاصله على ليسانس لغة عربية

أنثى مسلحة

البطاقة مارية حتى ٢٠٣١/٠٥/٠٨





Dms

← هبة

← مروة خمار + زهور

← مروة مصرية



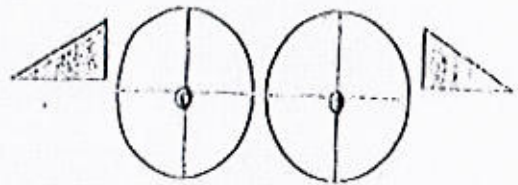
مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

المنشأة المحترمة
مستشفى
مصر

MDT FORM

Name	خديجة موكي عباس
Age	35
MRN	5.11.11.11.11.11
ID	111111111111
Co morbidity	2.5 / 2709

111111111111



Clinical Data

Bx:	SM:
IHC:	Mets Workup:
ER:	
PR:	
Her2:	
Ki67:	
T ₃ N ₁ M ₀	
COMMENT: ESS 617m exclusion of SM - no complete of metastatic no need	

MDT OUTCOME & PLAN

F. Ady = 617m for Ede Semara Zoladex	
--	--

SIGNATURE			
MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY

1339

Name : KHADIGA MOUSA ABBAS

Date : 06-01-2026 11:29

Branch : Portable

Medical No. : GRI

260106112923STD131221107523748902

Ref. Physician Prof :

Ins. Company

MRI OF THE BRAIN

Technique :

Axial T1, T2 ,FLAIR,

Coronal T2

Sagittal T2

Postcontrast T1

Findings:

- Right frontal intra axial SOL is seen measuring 18 x 14 mm eliciting low T1, iso intense T2 and bright FLAIR WI signal with contrast uptake. it is seen associated with surrounding edema and causing mass effect in the form of effacement of the adjacent cortical sulci.
- Mild dilatation of the ventricular system
- No midline shift or deformity
- No cerebellar or brain stem areas of altered MR signal.
- Normal cortical sulci
- No extra axial collections.
- No cerebello pontine angle masses.
- No gross posterior fossa abnormalities.\
- Partial empty sella.
- Prominent peri-optic CSF spaces bilaterally.

OPINION:

- *Right frontal SOL ? metastatic for MR spectroscopy.*
- *Mild involutional brain changes.*
- *Partial empty sella with prominent perioptic CSF spaces bilaterally raising the possibility of benign increased intracranial tension for clinical correlation.*

Patient Name	: الأستاذ / خديجه موسى عباس	Request No.	: 13003580
Patient No.	: 28808102301008	Request Date	: 23/11/2025
Path No.	: 3636/25	Verification Date	: 30/11/2025
Sex	: Female	Financial Category	: جوستاف روسي - نفقة الدولة
Age	: 37 Y	Department	: Out Patient
Referring Doc :		Registered by	: .marwa mohamed youss

Pathology Report

Clinical Data: Right parotid small solid mass lesion ? pleomorphic adenoma, with multiple submandibular, submental and upper cervical lymph nodes likely reactive inflammatory.
Nature of Specimen: Tru-cut biopsy, collected on 23/11/2025, at 3:10 pm, Cold ischemia time is 30 seconds.

Gross Examination:

- Two gray white tissue cores measured 1 cm & 1.5 cm long, totally submitted.

Microscopic Examination:

- Histopathological examination reveals cores of salivary tissue showing infiltration by tumor formed of mildly pleomorphic epithelial cells showing eosinophilic to clear cytoplasm, arranged in sheets, nests & cribriform like pattern with lumen containing eosinophilic material. No mitosis or necrosis in specimen received.

Diagnosis:

Right intra-parotid mass, Tru-cut biopsy:

Findings are in favor of low grade carcinoma.

- Case for further immunophenotyping including (P63, S100, DOG-1, ect.....).

Signature

Prof. Dr. Gamal Abd-El-Aty.

Dr. Mennan Hossam.

Verified by :

Dr.Duaa Mohamed

Printing Date : 04/12/2025 11:47:19

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غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name :	الأستاذة / خديجة موسى عباس	Request No. :	13003580
Patient No. :	28808102301008	Request Date :	23/11/2025
Path No. :	3636/25	Verification Date :	04/12/2025
Sex :	Female	Financial Category :	جوستاف روسي - نفقة الدولة
Age :	37 Y	Department :	Out Patient
Referring Doc. :		IHC No. :	
		Registered by :	.marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

CLINICAL DATA

Right small solid mass at the parotid with multiple submandibular , submental , upper cervical lymph nodes likely reactive inflammatory

NATURE OF SPECIMEN

Tru cut biopsy

Gross examination :

Technique:

Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:

S100 (Polyclonal), DOG-1, P63 (4A4).

MICROSCOPIC EXAMINATION

Examination revealed tumor cells , mixture of vacuolated cells , epidermoid cells , few cells with intracellular secretions , pushed nuclei ,surroundd by dense sclerotic stroma .No necrosis , minimal pleomorphism .No lymphovascular emboli.

IPT :

P63;Diffuse strong in tumor cells

S100:Negative

DOG1 :Negative

DIAGNOSIS

Right intra- parotid mass, tru cut biopsy :

Low grade malignant neoplasm favoring moderate grade mucoepidermoid carcinoma (P63 positive, S100 negative)

Signature

Verified by :

Dr.Heba Hasan

Printing Date : 04/12/2025 14:58:28

Page 1 of 2

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Patient Name	الأستاذة / خديجة موسى عباس	Request No.	13003580
Patient No.	28808102301008	Request Date	23/11/2025
Path No.	3636/25	Verification Date	04/12/2025
Sex	Female	Financial Category	جوستاف روسي - نفقة الدولة
Age	37 Y	Department	Out Patient
Referreing Doc.		Registered by	.marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Prof. Dr. Maha Guimei
Dr.Walaa Ibrahim Ghoneim

Verified by :
Dr.Heba Hasan

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Page 2 of 2

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

CAIRO UNIVERSITY
FACULTY OF MEDICINE
PATHOLOGY DEPARTMENT

Pathology No: 68f

Name خديجة موسى سليمان

Age: 34 Sex: female

Clinical diagnosis: Invasive duct carcinoma

Hospital no: 2880810230

Nature of specimen Left breast inferior margin

Department: 25b

Date: 14/07/21

PATHOLOGY REPORT

Gross

Received for frozen section:

- A sector of mammary tissue measured 9.5x5x4 cm with attached skin ellipse measured 9.5x4 cm. The deep surface showed wall of previous cavity. Serial sections revealed focus measured 1x1 cm with ecstasic spaces oozing pasty material. The focus was located just on deep- medial margin.
- Extra Deep - Fibrofatty and muscular tissue piece measured 4x2x1 cm.
- Extra medial - Fibrofatty tissue piece measured 6x4x2 cm.

MICROSCOPIC

Examination of sections from the received sector revealed a focus of invasive malignant growth formed of tubules and cords of moderately anaplastic epithelial cells with abundant eosinophilic cytoplasm. Surrounding foci of high grade DCIS were seen. Surrounding wide resection site changes in the form of fat necrosis and foreign body giant cell reaction were seen. The superior, lateral, inferior, medial, deep, extra inferior and extra medial margins were free of tumor involvement.

DIAGNOSIS

- Residual focus of invasive duct carcinoma, NST, Grade II, with high grade DCIS and previous resection site changes.
- Free resection margins.

DR. PASSANT ESSAM

DR. AYA MAGDY

Checked by: SM

Revised by: BY



Patient Name: Khadiga Mousa Abbas Seif	Date : 28/10/2023
Exam : MAMMOGRAPHY BOTH	Age : 35 Year(s)
Medical ID: 3659/2021	Pal. ID: 16804

RIGHT BREAST MAMMOGRAPHY WITH BILATERAL US

Indication:

Patient had left MRM

Technique

- right breast MLO and CC views
- Bilateral ultrasound

Report:

- Scattered fibroglandular densities of the right breast (ACR B)
- No spiculated masses or grouped microcalcifications.
- Complementary ultrasound also showed no right breast solid or cystic masses
- Normal skin thickness and nipple-areola complex, with average caliber of the retro-areolar ducts.
- Nonspecific right axillary LN, with preserved fatty hila and uniform cortical thickness.
- Free left operative bed with no evidence of recurrent mass lesions
- Free left infra and supra clavicular regions.

Opinion:

- Left MRM with free operative bed with no evidence of recurrent mass lesions
- No right breast suspicious findings (BIRADS 2)

Yours truly

Dr.Wafaa Ali



"HEIROCK"

Report

Patient Name:	Gadiga Mosa Abbas sef el-nasr	Patient Age:	33
Patient Code:	Q26936	DATE	2/10/2021 12:10:30 PM

^{99m}Tc MDP SKELETAL SCINTIGRAPHY

Clinical history

- A patient with history of left breast neoplasm.

Technique

- Anterior and posterior whole body scans were acquired 2 hours post IV injection of 740 MBq of the tracer.

Findings

- No abnormal osseous tracer uptake is seen that would account for bone deposits.
- Degenerative changes of the spine are noted.
- Osteoarthritic changes of the both knees are seen.

Opinion

- The surveyed skeleton is free from metastatic osseous lesions.

Dr. Marwa Abd Allah, MSc

Prof. Dr. Shereen Yassin, MD

جولف
روسلاني
9-11



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

مصر
جمهورية
مصر

09/13/2026

MDT FORM

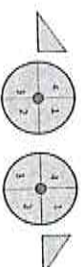
Name:	Khadija Mousa Abbass .	MRN:	1339
Age:	38 Y	ID:	28808102301008
Menopausal Sta		Phone:*	01142543518
NRS SCORE:		Marital Status:	
BMT:		Family History:	
Gender		Reason Of MDT:	
CO Morbidity:		Breast cancer	
Surgical History:*	na	ECOG Performance Status	
		00 10 20 30 4	

Clinical Data:

R

L

BX: IDC G4
ER: 7/8 **PR:** 8/8 **HER2:** +3 **KI67:** 30%



T N M

History:

38YEARS OLD FEMALE PT WITH LT BREAST CANCER METASTATIC TO BRAIN AND LEFT PAROTID RECEIVED NACT THEN BCS 6/2021 THEN LT MRM YPT1N2M0 (LN 5/14) 8/2021 ADJUVANT HERCEPTIN+PORT 21 Fx +

TAMOXIFEN

SINCE 10/2021 Then started femara zoladex 10/2025 SM 12/2025 FREE. MRI BRAIN 1/2026 Right frontal SOL metastatic for MR spectroscopy: RIGHT frontal lobe lesion 2 *1.4 cm suggestive of high

*grade neoplasm,Mild involutional brain changes.

Partial empty Sella with prominent petri optic CSF spaces bilaterally raising the possibility of benign increased intracranial tension for clinical

correlation.CT NECK and cap 1/2026

Right parotid enhancing soft tissue lesion measuring 15x12mm bilateral submandibular LNs noted; the largest is measuring 16 x 13 mm.Few tiny bilateral scattered pulmonary nodules LARGEST MEASURING 3.5 MM AT RT

UPPER LOBE. Hepatomegaly.

Few tiny left adrenal calcifications.

Right intra-parotid mass, Tru-cut biopsy: Findings are in Favor of low grade carcinoma. - Case for further immunophenotyping including (P63, S100, DOG-1

Sonomammography:

06/2026 : Patient underwent surgical excision of right frontal SOL (PATHOLOGY : METASTATIC DISEASE OF BREAST ORIGIN PATIENT RECEIVED 5 SESSIONS OF IMRT TO BRAIN

Biomarker/Molecular

Metis Workup:

Recommendations:

For:-Zometa every 6months
-the possibility of Xeloda/ Tykerb

MDT Outcome & Plan

-Zometa every 6months / new us parotid and lhc of bx
-start Xeloda/ Tykerb

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
Dr Nermeen Mostafa	د. نيرة مرسى		

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